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Name: إيمان عبدالمحسن عبده احمد National ID: 26302200103224	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ابن
عبد المحسن عبده احمد
اسم من فاسم السعدنى
الشرابية - القاهرة



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W4397736

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٢٠٣٣/٠٣ ٢١٣٠٢٠٠١٠٣٢٢٢
رَبِّهِ هَمَزَل

أرملة

مستلمة

انقش

البطاقة مصرية حتى ٢٠٣٠/٠٣/٠٢





MDT FORM 20 / 8 / 2025 G

<u>Name</u>	Dr. [illegible]	<u>MRN</u>	
		<u>ID</u>	114010
<u>Age</u>	61 yrs	<u>Phone</u>	1157757900
<u>Menopausal status</u>	Postmenopausal	<u>Marital status</u>	C 72.000.1.200
<u>NRS SCORE</u>		<u>Family history</u>	
<u>BMI</u>		<u>REASON OF MDT</u>	
<u>CO morbidity</u>	Stroke 2014		

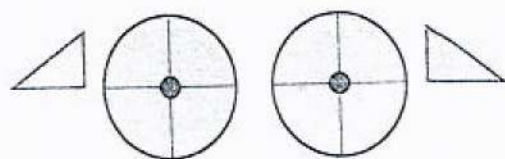
Clinical Data

BX: Pleomorphic lobular carcinoma

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<u>ER:</u> 8/8	<u>PR:</u> 4/8	<u>HER 2:</u> +3	<u>Ki67:</u>
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Sonomammography:

Mets Workup:

tree

TNM

Received: MACT + Anti Her2neu

Recommendations: Pt is on Adjunct Hormonal ~~TH~~, to continue as is for 7 yrs.

MDT OUTCOME & PLAN

DI OUTCOME & PLAN
To Continue on - Females
- Cerebrum
- Vit D

- 2 meter every 6 months.

SIGNATURE

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

MDT Form
(DOCTYPES-MDT_FORM4)



Ministry of Health
Central Health Laboratory
Pathology Department

Immunohistochemistry report

Report Number: m5059

Female

الجنس

التاريخ

التاريخ

23/09/2020 08:48

29/09/2020 09:56

تاريخ استلام العينة
تاريخ التقرير

العمر
57 Y

معدة مامبر
معدة مامبر

اسم المريض

الجنس

تاريخ أخذ العينة

ملاحظات التحليل

خاتمة

Primary Sample:

Paraffin Block

Test:

ER & PR & HER2 neu (ER&PR: Histo staining system - Her2neu: DAB staining system)

Comment:

Technique:

Sample received was treated by: CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody - CONFIRM anti-progesterone receptor (PR) (1E2) Rabbit monoclonal primary antibody.
Sample received was treated by: VENTANA anti-HER2/neu (1B6) Rabbit monoclonal primary antibody using DAB Detection Kit on Ventana Benchmark GX.

Result:

ER: strong nuclear staining of 80 % of tumour cells
PR: moderate nuclear staining of 5 % of tumour cells
HER2/neu protein over expression: strong continuous membrane staining of > 10 % of tumour cells.

Referral Lab:

Comment:

Conclusion:

ER: POSITIVE (SCORE 3/8)
PR: POSITIVE (SCORE 4/8)
HER2/NEU PROTEIN OVER EXPRESSION: POSITIVE (SCORE 3+) (CORE BIOPSY)

Lab Director: د/ فريز مامبر

Pathologist:

د/ فريز مامبر
مدير المختبر
المختبر المركزي
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يتم الاعتماد بأصل النتيجة لمدة 30 يوم من تاريخ استلامها

Page 1 of 1



Patient Name: Eman Abd Al Mohsen Abdou Ahmed	Date : 20/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 62 Year(s)
Medical ID: 4175/2020	Pat. ID: 10880

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM in

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

20/03/2025 2:04 PM



Patient Name: Eman Abd Al Mohsen Abdou Ahmed	Date : 04/12/2024
Exam : C.T CHEST	Age : 62 Year(s)
Medical ID: 4175/2020	Pat. ID: 10880

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST:

As compared to the last available study dated 20/04/2024, the current study shows:

- Left mastectomy with clear operative bed.
- **Rather stationary course** regarding the previously seen:
 - Relatively reduced volume of left lung.
 - Partial collapse and cystic bronchiectatic changes of the lingula.
 - Left basal calcified pleural plaque.
 - Few prominent mediastinal (retrocaval and aorto-pulmonary) lymph nodes.
- Enlarged cardiac size; for echocardiography correlation.
- No evidence of pleural effusion or masses seen.
- No pericardial effusion detected.
- Multiple aortic calcified atheromatous plaques.
- **Still noted** enlarged heterogeneous thyroid lobes with right sided calcific focus for ultrasound correlation.
- Dorsal spondylosis.
- Upper abdominal cuts show GB stones and para-aortic tiny lymph nodes.

OPINION:

- **Status follow up showing stationary course since last dated study.**

Dr. Yasser Rashad, MSc

Signature Prof. Ibrahim Elsedawy
29/01/2025 7:23 PM

Patient Name: Eman Abd Al Mohsen Abdou Ahmed	Date : 20/04/2024
Exam : MRI BRAIN	Age : 61 Year(s)
Medical ID: 4175/2020	

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T2WIs

MR Findings:

- Bilateral deep white matter periventricular sheets and subcortical of areas of abnormal bright T2 and FLAIR signal are seen with multiple bilateral periventricular and subcortical white matter small lacunar infarcts.
- No definite intra cranial masses on non-contrast base.
- No signs of intra cranial bleeding.
- Mild bilateral symmetrical dilatation of the ventricular system.
- Prominent cortical sulci, sylvian fissures and basal cisterns.
- No mid line shift.
- Relative decrease in size of the brain stem and cerebellum with small pontine bright T2 foci.
- Intact cervicomedullary junction.

Opinion:

- Brain involutinal changes.
- Subcortical arteriosclerotic leukoencephalopathy with multiple bilateral cerebral lacunar infarcts, Fazekas-II.
- No definite worrisome intra-cranial masses on non-contrast base, for further evaluation by contrast enhanced study if clinically indicated

Much Obliged,
Dr. Sherif Sameh, M.D.



Patient Name: Eman Abd Al Mohsen Abdou Ahmed	Date : 08/01/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 61 Year(s)
Medical ID: 4175/2020	Pat. ID: 10880

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- average in size (about 15 cm at the right MCL), bright coarse echopattern. No focal lesion could be seen.
No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- contracted upon multiple stones in side , CBD is not dilated.

Spleen: -

- average size, displaying homogenous echopattern.

Kidneys: -

- Are of normal size , site , with no gross stones or backpressure changes ,
Mild increase in echogenicity
simple right renal upper zone cortical cyst = 15 mm in diameter.

Para-aortic and pancreatic area: no gross lesion. -

No ascites .

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus : unremarkable
adenxal mass or cyst.

Opinion:

Parenchymal liver disease.
Bilateral renal mild increase in echogenicity more at the right side (grade I nephropathy) for laboratory correlation
Chronic calcular cholecystitis.
Right renal cyst.
Colonic gaseous distention.

Signature Dr . Nermien Ramadan

08/01/2024 4:45 PM

Patient Name: Eman Abdel-Mohsen

Date: 26/12/2021

^{99m}Tc - ^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Unfortunately, the previous bone scan film is not available for comparison, however, in comparison to previous report the current scan revealed:
- No newly developed since last study.
- No appreciable change could be detected since last study
- Still noted;
- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected alloyer the surveyed skeleton.
- Both kidneys are seen.